the low caseload to inadequate testing. By the last week of March, India was testing about 31 people for every COVID-19-positive case. As a matter of comparison, the US was testing about nine, South Korea about 38, Taiwan about 111 and Russia about 200. In total, we were testing about one person in every one million people, but our test positivity rate was 2.10 per cent.[22] According to WHO guidelines at the time, ‘a general benchmark of 10% or even better, lower than 3% could be considered a sign of adequate testing’. However, all experts at the time opined that higher the testing, better the possibility of successfully tracing contacts and thus, higher the chance of curbing the spread of the infection. India had to depend on importing testing kits mostly from China to fulfil its requirements. Due to every country in the world scrambling to source COVID-19-fighting essentials from China and the other limited Asian countries where they were available, demand was huge and largely unmet. Often, products of compromised quality would be provided, leaving the countries in a lurch at an extremely sensitive time. India too, early on, received about five lakh faulty testing kits from China.[23] While the two countries tried to discuss the issue through diplomatic channels, the damage on ground was that several false negatives were marked. Those individuals were let out into the community, assuming they didn’t carry the infection, when in fact they could have been potentially spreading the infection to tens and hundreds of people.

Over the past six weeks, a group of nine women, many of them mothers and one gentleman led by an eight-month pregnant virologist Minal Dakhave Bhosle worked tirelessly in the laboratories at Mylab Discovery Solutions in Pune trying to find a cheaper, Indian alternative to the imported testing kits which India was relying upon.[24] This group had been actively involved in making PCR kits for several other diseases for several years. However, the usual timeline of four to six months to come up with a test kit was just not acceptable to them anymore. Since mid-February, the team was determined to dive into their experience of developing Deoxyribonucleic acid (DNA) and RNA tests for applications in clinical, veterinary and food safety settings and connecting with researchers who had worked on the SARS family of viruses in India. The WHO guidelines for COVID-19 tests came in handy as the team put together the reagents they would need to extract nucleic acids from patient’s saliva samples in order to carry out real-time PCR. They even gathered material they would require to use as